



MAGETHI
INDUSTRIAL PSYCHOLOGY SERVICES

LOSS OF EMPLOYABILITY AND LOSS OF EARNINGS

MEDICO-LEGAL REPORT

FOR

JACOB THABISO MOTAUNG

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Full Name	Jacob Thabiso Motaung
Date of Birth	15 November 1964
Identity Number	641115 5491 080
Age	61 years and 05 months
Gender	Male
Date of Accident	26 November 2024
Age at the time of the accident	60 years
Education level	Grade 11
Occupation	Pre – accident: General Worker Post- accident: Unemployed
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Your Reference	Chuene/mva/20795
Our Reference	MIPS/2026/MOTAUNG
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HPCSA registration	PS 0122220
Capacity	Industrial Psychologist
Date of Assessment	12 January 2026
Date of Report	30 April 2026

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1. PURPOSE OF ASSESSMENT

The purpose of this assessment is to determine the extent and impact of the accident-related injuries on the client's physical and cognitive functioning in order to predict future career-related prospects and earning potential.

Conclusions and recommendations made in the report are based on the results from an interview conducted with the client, behavioural assessment, psychological, medical and Para-medical reports that were made available to the writer.

2. SUPPORTING BACKGROUND INFORMATION

The following documents were made available at the time of assessment:

- Instruction letter from Chuene Attorneys dated 12/01/2026
- Copy of RAF 1 form completed by Dr. I.H. Chauke
- Copy of hospital records
- Copy of Orthopaedic Surgeon report compiled by Dr. S.K Mafeelane (date of examination: 12/01/2026)
- Copy of Cardiothoracic Surgeon report compiled by Prof. R.S Chauke (date of interview: 14/04/2026)
- Copy of Narrative Test report compiled by Prof. R.S Chauke (date of report: 22/04/2026)
- Copy of Neurologist report compiled by Dr. B. Ngele (date of interview: 13/01/2025)
- Copy of Clinical Psychologist report compiled by Dr. T Sibanda (date of examination: 13/01/2026)
- Copy of Occupational Therapist report compiled by L. Tau (date of consultation: 12/01/2026 & date of report: 25/04/2026)

3. DESCRIPTION OF ASSESSMENT METHODS USED

Structured Interview Questionnaire: The claimant was interviewed using a questionnaire to obtain information on the following aspects of his life:

- i. Personal information
- ii. Educational background

- iii. Family background
- iv. Physical wellbeing
- v. Level of independence
- vi. Social/support systems
- vii. Level of aspirations

Behavioural Assessment: The interview tool was used to assess for post morbid behavioural change.

Psychological Assessment: A psychological assessment was not administered.

4. CLIENT PERSONAL FACTORS

4.1. Client

Mr. Jacob Thabiso Motaung was born on the 15th of November 1964, making him 61 years and 05 months at the time of report writing. He was involved in a Motor Vehicle Accident (MVA) on the 26th of November 2024 at the age of 60 years. In this report, he will be referred to as Mr Motaung.

Impression: Mr. Motaung attended the interview alone; he was well groomed and well attired. Rapport was established between the assessor and Mr Motaung and the interview was conducted in an African vernacular. His thought processing and recollection of events were deemed to be fair. In terms of his physical appearance, his feet appeared to be swollen, and he had flip flops on.

4.2. Family Background

Mr. Motaung reported he was raised by his late parents, and his wife constitutes his social support. He indicated that he was the fifth of twelve children from his parents' relationship; he noted that nine of his siblings as well as one of his children are deceased. Moreover, he shared that he is married and has two children.

Details of Mr. Motaung's family are presented in the table below

Table: family and dependent information

Name	Age	Relationship	Qualification	Work situation	Any other info
Philisiwe Motaung	Unknown	Wife	Grade 12	Unemployed	Has 3 children
Tumelo Mabasa	32	Daughter	Grade 12 & vocational qualifications	Teacher	Has 1 child
Tebogo Mabasa	25	Son	Grade 11	Unemployed	-

4.3. Socio-Economic status

Mr. Motaung reported that he is currently unemployed and is financially dependent on an old age pension grant. He shared that he resides in a two-room brick house located at Ladysmith, KwaZulu Natal Province. He indicated that he owns the home which is occupied by six family members. Regarding amenities, he mentioned that they have adequate access to electricity and water supply.

4.4. Health Status before and After Accident

Pre-morbid, Mr. Motaung reported that he was in a good health with no known chronic illnesses. He narrated that he was involved in a pedestrian vehicle accident when he was young and does not recall the date; he noted that he did not sustain any injuries from the accident. He stated that was able to conduct all household duties.

Post-morbid, he narrated that he prefers light household duties. He elaborated that he experiences pain on his neck, right shoulder, back, rib cage, chest and right leg exacerbated by cold weather, he further stated that his feet swell and he struggles to wear closed shoes as well as to lift and carry heavy objects. He noted that he takes pain management medication that he collects from a local clinic.

4.5. Educational History

- In terms of schooling, Mr. Motaung informed that he completed Grade 11 level of education at Breydam High School in 1984. He stated that he discontinued school after failing Grade 12 in 1985.
- In 1986, he obtained a Bricklaying Certificate at Harrismith Training School, duration of the course was three months.

- Lastly, he shared that he did not acquire computer skills and is in possession of a C1 driver's license which was issued in 2012 and is currently expired.

Details of Mr Motaung's educational history are presented in the table below.

Table: Schooling Records

Name of school	Year	Grade	Result
Breydam High School	1984	Grade 11	Passed
Note: The writer was not in possession of a National Senior at the time of report-writing.			

Table: Mr. Motaung's Vocational Training

Institution	Qualification	Date
Harrismith Training School	Bricklaying Certificate	1986
Note: The writer was not furnished with certificate at the time of report writing.		

4.6. Claimants' Employment Profile

4.6.1. Employment History: Pre-morbid

- In terms of employment, Mr. Motaung informed that he commenced working in the capacity of a General Worker at Port Natal Municipality in 1986. He stated that he was employed on a contract basis and worked every Monday to Friday from 07:30 – 16:00. He stated that he was remunerated R350.00 per month and his duties included assisting with building of a school and community hall. He received training to effectively perform his tasks and he was uncertain of career progression opportunities within the municipality. Moreover, he indicated that he ceased employment after six months to seek better prospects.
- In 1987, he commenced conducting piece jobs working on various projects as a Bricklayer. He stated that he worked for various employers and worked flexible days and hours for an average of twenty days a month. He indicated that he was remunerated R150.00 per day and his duties included assisting with the plan reading and building of houses. In addition, he narrated that there

were career no progression opportunities within his line of work, he however ceased working in this capacity after securing better prospects in 1988.

- He indicated that he was employed at NBR Construction located in Belfast in 1988. He indicated that he was employed as a Bricklayer on a contract basis and worked every Monday to Friday from 07:00 – 16:00. He noted that his duties included building of hostels and accommodation at a mine and earned approximately R3 500.00 per fortnight. His certification reportedly served as training to effectively perform his tasks. He stated that there were career advancement opportunities, and his work performance was deemed to be good. In addition, he stated that employment ceased when his contract ended after six months.
- From 1989 – 1996, Mr. Motaung indicated that he was conducting piece jobs for various employers. He recounted that he worked as a Bricklayer and does not recall how many projects he secured per month as well as his income. He further stated that his duties included assisting with building of houses within his community and his work performance level was fair. He noted that there were career progression opportunities within his line of work, he however ceased working in this capacity when he secured better prospects.
- He stated that he was employed as a Bricklayer for To-See-Construction company from 1997. He stated that he was employed on a contract basis and worked approximately six days a week from 07:00 – 17:30. He indicated that he was remunerated approximately R12 000.00 per month and his duties included building of houses and walls. He stated that his tasks and his work performance was fair and he was uncertain of career progression opportunities within the company. He further noted that he received training to effectively perform his tasks. Lastly, he expressed that employment ceased after seven months when his contract ended.
- According to Mr. Motaung, he secured employment as a Bricklayer at Siyajuluka Training Construction in 1997. He was reportedly employed on a contract basis and worked every Monday to Saturday from 07:00 – 16:00. He further noted that his duties included building of houses and community halls as well as renovate schools; he earned approximately R350.00 per day. His certification served as training to effectively perform his duties and his work

performance level was deemed to be satisfactory. Furthermore, he recounted that there were no career progression opportunities and employment ceased owing to his contract ending in 2020 as result of the covid pandemic.

- Moreover, Mr. Motaung recounted that he was employed at Soyguba Construction from 2022 – 2023. He stated that he worked every Monday to Friday from 07:00 – 16:00. He stated that he was employed as a Bricklayer on a contract basis and he was remunerated R400.00 per day. He recounted that his duties included building of houses around the community. He indicated that his work performance was good. He noted that he was uncertain of career progression opportunities and employment ceased when his contract ended.
- From 2024, he narrated that he was employed as a General Worker and Team Leader by Mr. Magubane and worked in Ladysmith. He stated that he was employed on a contract basis and worked every Monday to Friday from 07:00 – 17:30. He stated that his duties included utilising machinery to repair the road and patch potholes and he earned R350.00 per day. He expressed that he received informal training to perform his tasks and there were no career progression opportunities within his line of work.
- At the time of the accident (26 November 2024) Mr Motaung reported that he was employed by Mr Magubane in the capacity of a General Worker and Team Leader.

4.6.2. Employment History: Post-morbid

- Subsequent to the accident, Mr. Motaung reported that he was hospitalised for approximately one month and recuperated at home for another month. He indicated that he was not remunerated whilst in recuperation and his family cared for him.
- Mr Motaung reported that he did not return to work as he would not be able to cope with work demands; as a result of his inability to lift and carry heavy objects due to pain on his back, rib cage, chest, neck, right shoulder and right leg exacerbated by cold weather.
- At the time of finalising the report, Mr Motaung reported that he remains unemployed and not actively seeking employment.

4.6.3. Collateral information

The writer contacted Mr Magubane, former employer on 083 961 9240, to confirm Mr Motaung's pre-morbid employment as well as work performance, he confirmed the following:

- Mr. Motaung commenced working in the capacity of a General Worker and Team Leader employed on a contract basis for a project that was for a duration of six months.
- His duties included supervising subordinates and ensured that target are met as well as assist with the repairing and patching of potholes using machinery.
- He was unable to confirm earnings due to the POPI Act.
- His work performance and interpersonal skills were deemed to be satisfactory.
- There were no career progression opportunities.
- He is aware of Mr Motaung's involvement in an accident as the matter was reported to him.
- According to Mr. Magubane, Mr Motaung reported that he would not be able to return to work as he would not cope with work demands for the remainder of the project which ended in February 2025.
- Retirement age is unspecified.

Details of Mr. Motaung's pre-and post-morbid employment history below

Table: Detailed Employment History

Company	Position	Date	Salary	Reason for leaving
Port Natal Municipality	General Worker	1986 (6 months)	R350.00 per month	Resigned
Various employers	Bricklayer	1987 - 1988	R150.00 per day	Secured better prospects
NBR Construction	Bricklayer	1989 (6months)	R250.00 per day	Contract Ended
Conducting piece jobs	Bricklayer	1989 – 1996	Unknown	Secured better prospects
To See Construction	Bricklayer	1997 (7 months)	R12 000.00 per month	Contract ended
Siyajuluka Training Construction	Bricklayer	1997 – 2020	R350.00 per day	Contract ended (Covid 19)

Soyguba Construction	Bricklayer	2022 - 2023	R400.00 per day	Contract Ended
Mr Magubane	General Worker and Team Leader	2024 – TOA	R350.00 per day	Accident Occurred & did not return
Note: Proof of earnings was not received at the time of report writing				

4.7. Career aspiration

Pre– morbid, Mr. Motaung reported that he aspired to own a construction company.

Post- morbid, he stated that he feels unfit to work.

5. THE ACCIDENT

5.1. Description of Accident by the Client: Mr. Motaung

Mr. Motaung reported that he was involved in a Motor vehicle accident on the 26th of November 2024; he was a front seat passenger. He shared that he does not recall details leading to the accident. He reportedly lost consciousness at the scene and regained consciousness whilst in hospital. According to Mr Motaung he was transported by an ambulance to Ladysmith Hospital where he received medical intervention for approximately one month; he noted that he recuperated at home until end of January 2025; his family cared for him.

5.2. Injuries Sustained: according to Orthopaedic Surgeon report compiled by Dr. S.K Mafeelane (page 4 of report) the following is noted:

The clamant sustained the following injuries:

- Right neck of humerus fracture
- Bilateral pneumothorax- defer to cardiothoracic surgeon.
- Right foot injury
- Head injury- defer to neurosurgeon

He is right hand dominant.

5.3. Current Complaints (Subjective)

- “I experience neck pains”.

- “I experience pain on my chest and rib cage”
- “I experience on my right shoulder and back”.
- “I experience pain and swelling on my right leg exacerbated by cold weather conditions”.
- “I struggle to lift and carry heavy objects”
- “I struggle to wear closed shoes”

5.4. Behavioural and social complaints

- “I become reserved”.
- “I have become short tempered”

6. NATURE OF INJURY AND SUMMARY OF EXPERT REPORTS

The summary below is compiled from the documents submitted as indicated in Section 2 of this report. The writer has noted the following information as important in evaluating Mr. Motaung’s future occupational role:

According to Orthopaedic Surgeon report compiled by Dr. S.K Mafeelane

Page	Comment
8 – 11	Special Investigation X-rays were done by Drs. Mkhabele and Indunah Inc. on 12 January 2026 and were reported as follows: Linear lucency with sclerotic margins and marginal osteophytes in the neck of the humerus. There is associated bony proliferation noted around the old fractures line. The humeral head is rotated clockwise within the glenohumeral joint. Significant cortical irregularity and bony proliferation noted in the base of the 5th metatarsal bone suggestive of chronic injury.

	Joint space narrowing of the cuboideonavicular and 4th metatarsophalangeal joint suggestive of osteoarthritic (secondary) changes. Pain and Suffering Acute Pain He reports that he had acute pain that was present for almost 10 to 14 days as result of the injuries. The claimant required analgesics to ease pain. Moderate pain This type of pain most likely lasted for almost 3 weeks while he continued with analgesics. Chronic pain He continues to suffer the inconvenience and discomfort from the injured areas. Scarring and Disfigurement Scars as a result of the accident have been noted. Impact of Injury He has great difficulty with prolonged walking. He has great difficulty carrying and lifting heavy objects. He has great difficulty doing household chores. He has great difficulty doing overhead activities.
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	He has great difficulty lying in the injured areas. Life Expectancy Injuries will not shorten his life expectancy. Disability and Impairment Using the combined values chart on page 604, he has 12% whole person impairment. The injuries sustained have resulted in serious long-term impairment. Retirement and Employability He is a pensioner.
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According to Cardiothoracic Surgeon report compiled by Prof R.F Chauke

Page	Comment
No page numbers allocated	Damages - Mr Motaung sustained acute thoracic injury–related morbidity for a period of at least three (3) months following the accident. The quantification of acute-phase damages (including hospitalisation, ICU care, and immediate treatment costs) falls outside the scope of this report. - Mr Motaung is expected to continue experiencing intermittent episodes of chest pain and generalised musculoskeletal pain, attributable to the injuries sustained in the accident. - It is well established in medical literature that rib fractures and associated thoracic trauma are frequently linked to the development of chronic chest wall pain, which in some cases may persist long-term or even lifelong.

	- A range of treatment modalities exists for the management of chronic posttraumatic chest pain, including: - o Pharmacological management (analgesics) - o Physiotherapy and rehabilitation - o Interventional and, in selected cases, surgical options However, current evidence does not demonstrate clear superiority of one treatment modality over another, and management is typically individualised based on patient response and symptom severity. Mr Motaung is currently responding adequately to conservative management with analgesics, and it is recommended that he continues to use these medications as required for pain control. Loss of Amenities Mr Motaung experienced a significant loss of amenities of life for a period of at least three (3) months following the accident, attributable to the severity of his chest injuries, prolonged hospitalisation, and recovery period. During this time, his ability to engage in normal daily activities, physical functioning, and social participation would have been markedly impaired. • On a balance of probabilities, Mr Motaung is expected to experience ongoing and intermittent loss of amenities of life in the future, as a result of chronic chest pain and associated functional limitations. • These limitations are likely to include: - o Reduced tolerance for physical exertion - o Difficulty with activities requiring upper body strength or endurance - o Interference with recreational, social, and occupational activities
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	• The persistence of pain and functional restriction is consistent with chronic post-traumatic thoracic injury, which is well recognised to have a long-term impact on quality of life. Prognosis and Life Expectancy The prognosis is good. Life expectancy is not expected to be affected by chest injuries sustained following the accident. Loss of work capacity Mr Motaung experienced a temporary total loss of work capacity for a minimum period of three (3) months following the accident, attributable to the severity of his thoracic injuries, ICU admission, and subsequent recovery phase. - During this period, he would have been medically unfit to perform any form of occupational duties, particularly those requiring physical exertion, due to pain, reduced respiratory function, and general deconditioning. - On a balance of probabilities, Mr Motaung is expected to experience intermittent future loss or reduction of work capacity as a result of chronic chest pain and associated functional limitations, likely secondary to underlying rib fractures and thoracic trauma. - These limitations may manifest as: - o Reduced ability to perform physically demanding tasks - o Decreased endurance and productivity - o Need for periodic absenteeism during pain exacerbations • The extent of future occupational impact will depend on the physical demands of his occupation, with greater impairment expected in manual or physically intensive work environments.
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	• Overall, Mr Motaung demonstrates a reduced work capacity compared to his pre-accident baseline, with a risk of ongoing episodic incapacity attributable to accident-related injuries.
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According to Narrative Test report compiled by Prof R.F Chauke

Page	Comment
No page numbers allocated	WPI Pulmonary Dysfunction: from possible multiple bilateral rib fractures resulting in chronic chest pains= 17%. Recommendation It is recommended that Mr J.T Motaung be compensated for moderate chronic chest pains from possible bilateral rib fractures. WPI is 17%.

According to Neurologist report compiled by Dr. B Ngele

Page	Comment
20 – 24	Impact on Education Mr Thabiso Jacob Motaung completed Grade 11 prior to the accident and was not engaged in further formal education at the time. Neurologically, although he sustained a mild traumatic brain injury with ongoing headaches and sleep disturbance, there is no objective evidence of significant cognitive impairment. As such, the accident has not directly affected his educational attainment, but persistent pain, fatigue, and reduced tolerance for sustained effort may indirectly limit his ability to participate in future training or skills development.

Impact on Employment

Mr Thabiso Jacob Motaung was employed as a construction general worker prior to the accident, a role requiring heavy physical labour. Since sustaining polytrauma, including a mild traumatic brain injury and significant musculoskeletal injuries, he continues to experience persistent headaches, sleep disturbance, and chronic pain affecting the back, chest, neck, right shoulder, and right lower limb. Although neurologically stable with no focal deficits, his reduced endurance, impaired mobility, and inability to lift or tolerate prolonged standing or driving prevent him from meeting the physical demands of his previous work. He has therefore been unable to return to employment and remains unemployed, with his functional work capacity significantly compromised by ongoing post-accident sequelae.

Neurological and Functional Outlook

Mr Thabiso Jacob Motaung has shown a stable neurological recovery following his mild traumatic brain injury, with preserved cognition, intact cranial nerves, and normal motor and sensory function, and no evidence of permanent focal neurological deficit or progressive decline. However, he continues to experience persistent post-traumatic headaches and sleep disturbance, which may remain chronic and affect his endurance and quality of life. Functionally, his outlook is mainly limited by chronic musculoskeletal pain and physical restrictions from associated polytrauma, resulting in reduced mobility, an antalgic gait, and diminished tolerance for strenuous activity. Overall, while his neurological prognosis is favourable, his functional recovery remains incomplete due to ongoing pain-related disability and reduced capacity for heavy manual work.

From a neurological perspective, his presentation is most consistent with a mild traumatic brain injury (concussion). Neurological monitoring during admission documented a Glasgow Coma Scale of 15/15 following stabilisation, and no evidence of severe intracranial complication has been identified in the available records. Clinically, he remains cognitively intact, awake, alert, and fully orientated, with preserved cranial nerve function and

	normal motor and sensory examination. There is no objective evidence of permanent focal neurological deficit, seizures, or progressive neurocognitive decline. Despite this neurological stability, Mr Motaung continues to experience persistent posttraumatic symptom burden, particularly recurrent headaches and sleep disturbance, which are commonly observed following concussion and may contribute to reduced endurance and quality of life. These symptoms are compounded by extensive musculoskeletal injuries, including multiple rib fractures with flail chest and pneumothoraces requiring intercostal drainage, a right humerus fracture, and injuries involving the right lower limb. Chronic pain affecting the lower back, chest wall, neck, right shoulder, and right leg remains his dominant ongoing complaint and is associated with reduced mobility and a mild antalgic gait. Functionally, the claimant remains independent in basic self-care activities such as feeding and bathing, but he is unable to perform more demanding instrumental tasks requiring lifting, carrying, prolonged standing, walking, or extended driving. His occupational capacity has been significantly compromised, as he was previously employed as a construction general worker and has been unable to return to heavy manual labour due to persistent pain, reduced endurance, and physical limitations. In summary, while Mr Motaung's neurological prognosis is generally favourable with preserved core neurological function following mild traumatic brain injury, his overall recovery remains incomplete due to chronic post-traumatic headaches, sleep disturbance, and substantial musculoskeletal pain-related disability. These ongoing sequelae continue to impair his quality of life, daily functioning, and employability in physically demanding work environments.
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	Prognosis & Life Expectancy Mr Thabiso Jacob Motaung's neurological prognosis is generally favourable following a mild traumatic brain injury, with intact cognition, normal cranial nerve function, and no evidence of permanent focal neurological deficit or progressive decline. However, he continues to experience persistent post-traumatic headaches and sleep disturbance, which may remain chronic and affect quality of life. Functionally, his long-term outlook is more limited by chronic musculoskeletal pain and physical restrictions from associated injuries, resulting in reduced mobility, endurance, and inability to return to heavy manual work. Overall, while neurological stability is expected, his functional recovery remains guarded due to ongoing pain-related disability. Risk Complications Mr Thabiso Jacob Motaung has a low risk of major neurological deterioration, with no evidence of seizures, focal deficits, or progressive cognitive decline. However, he remains at risk for persistent post-concussive symptoms such as chronic headaches, sleep disturbance, and cognitive fatigue. Ongoing musculoskeletal injuries and multiregional pain increase the likelihood of chronic pain syndromes, deconditioning, and prolonged functional disability. He is also vulnerable to secondary psychological complications, including anxiety and low mood, related to chronic pain and reduced occupational capacity.
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According to Clinical Psychologist Report compiled by Dr. T Sibanda

Page	Comment
35 – 37	Summary and Conclusions His presentation is most consistent with a mild traumatic brain injury with ongoing post-concussive symptom burden, particularly persistent headaches and sleep disturbance.

	The claimant's predominant ongoing disability is driven by chronic musculoskeletal pain arising from his associated orthopaedic and thoracic injuries. He has persistent pain involving the chest wall, lower back, neck, right shoulder, and right lower limb continues to impair mobility, endurance, and functional capacity, resulting in a mild antalgic gait and inability to perform heavy or sustained physical tasks. Although he remains independent in basic self-care, he is unable to perform more demanding instrumental activities and has been unable to return to employment as a construction general worker. Overall, the claimant's neurological prognosis is generally favourable with preserved core neurological function. However, his functional recovery remains incomplete due to chronic pain-related disability and persistent post-traumatic symptoms. Long-term multidisciplinary management, including rehabilitation, chronic pain control, and supportive psychological care, will be important in optimising his quality of life, maintaining independence, and reducing the risk of chronic disability. The neuropsychological symptoms present were headache, problems with balance, poor coordination, dizziness, visual problems, fatigue, sensitive to light, tinnitus and numbness. The cognitive symptoms present were cloudy, feeling slowed down, concentration problems and difficulty to remember. The emotional symptoms present were irritability, sadness, nervousness and fluctuating mood. The irregular sleep patterns were difficulty to fall asleep and sleeping less than usual.
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	Subjective symptoms indicated suggests possible neuropsychological deficit. The subjective intensity of the symptoms ranged from mild to severe. The PTSD symptoms were present. The subjective intensity of the symptoms ranged from mild to severe. The neuropsychological tests done using CNS Vital Signs battery of assessments above confirm the assessment findings, which concludes that there is persistent symptoms of neuropsychological deficit across several domains. The Neurocognition Index (NCI) providing the overall neuropsychological performance is low, denoting likely neurocognitive impairment and deficit. On the basis of information obtained, we conclude that his poor performance may be attributed to a combination of neurocognitive deficits due to traumatic brain injury, psychological stress and pain interfering with body general body functioning. The symptom inventories indicate that the claimant experiences several of the symptoms ranging from mild to severe degree. Overall, the concussion symptoms were rated as moderate to severe. THE QEEG is inclined towards deviation from the norm indicating neurocognitive deficits. In the assessment, the claimant reported among other experiences, low self esteem, anxiety and depression. Taking all of the above, I recommend that the claimant should be compensated for Moderate Neuropsychological Disorder. In addition to the head injury, I recommend that the claimant should be compensated for any other body injuries and psychological difficulties resulting from this accident which may be a possible source of further psychological distress. If compensation is granted, I recommend that funds
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	should be set aside for 20 sessions of psychotherapy. The goal of the psychotherapy will be to address the claimant's psychological distress. I expect that the recommended psychotherapy has a reasonable chance of success.
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According to Occupational Therapist Report compiled by L Tau

Page	Comment
27 – 30	Functioning in ADL/IADL/Leisure/Social The claimant reported that he experiences chest pains when taking a deep breath and during physical activity. As a result, he experiences difficulties walking for long when accessing amenities in his community. It must be noted that chest pains and shortness of breath will lead to decreased stamina in physically demanding tasks. The Occupational Therapy physical evaluation findings indicated that the claimant presented with altered functional usage of the right upper limb as he presented with restricted active and passive range of movement in the right shoulder, decreased muscle strength in the right shoulder muscle group, decreased grasp strength in the right hand, and bilateral manual handling of loads in excess of 9kg placed additional strain on his right shoulder. It must be noted that the claimant is right hand dominant. The claimant reported right foot pain when walking for prolonged periods, which was corroborated during the evaluation. It is noted that the claimant has a pre-existing condition resulting in leg discrepancy of 3cm, with associated asymmetrical posture and pre-existing balance impairments. The accident-related right foot injury has exacerbated his underlying biomechanical dysfunction, contributing to his antalgic gait pattern, decreased standing endurance, and decreased walking endurance. This combination of pre-existing and accident-related factors limits his community mobility, ability to stand in long queues during shopping, and capacity for any occupation requiring prolonged ambulation or weight-bearing through the lower limbs.

	The psychosocial evaluation revealed that the claimant presented with accident-related fear and anxiety. The writer defers to the opinion in Dr. Thabani Sibanda, Clinical Psychologist's Neuropsychological medico-legal report, dated 23rd January 2026 for further comment and recommended treatment. The writer is therefore of the opinion that as a result of injuries sustained in the accident in question, the claimant experiences sleep disturbances and limitation in the execution of instrumental activities of daily living. Work Potential The claimant first entered the employment market in the year 1986 as a Bricklayer Assistant at Port Natal Municipality. He reported that from the year 1987 until September 2024, he worked as a Bricklayer for various construction companies. Vocational analysis of this occupation indicated that it could be classified as a medium to heavy strength requirement type of work. At the time of the accident in question, from October 2024, the claimant was employed as a General Worker for Mr Magubane. Vocational analysis of his occupation indicated that it required frequent standing and walking, bilateral hand use, repetitive arm movements, lifting and carrying, as well as repetitive or sustained below knee level reach, and above shoulder level reach. The claimant reported that post-accident, due to injuries sustained, he never returned to work and has never been able to secure any formal or informal employment since. At the time of the evaluation in January 2026, he was an unemployed Pensioner. The writer is of the opinion that this outcome is conceivable, as due to ongoing symptomology in his right upper limb, right foot, chest region, recurring headaches, as well as difficulties with bilateral manual handling of medium to heavy loads, the claimant does not possess the functional capacity to execute occupation as a General Worker on a construction site. The writer is of the opinion that the claimant presented with the residual work
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	capacity to engage in sedentary to light strength requirement workloads, with the exclusion of frequent to constant above shoulder and overhead level reach action, as well as exclusion of frequent to constant below knee level work. He is suitable for unskilled to semi-skilled occupations, operating in an accommodative work environment, where reasonable accommodations can be provided for his physical challenges and cognitive deficits. It must be noted that most occupations that suite his strength capacity are administrative in nature or are of a supervisory role. When considering his low level of education and lack of vocational exposure to administrative work, it can be expected that he will not be able to secure such employment. It must however be noted that he is now of pension age and not expected to re-enter the employment market. Loss in Earning Capacity The claimant is a Pensioner and not expected to re-enter the employment market. When considering his age, low level of education, lack of formal skills training, as well as his residual work capacity, it can be deduced that his opportunity to engage in post-pension income generating activities has been curtailed. The writer is therefore of the opinion that from a functional perspective due to accident-related injuries, the claimant has suffered a decline in earning capacity. The writer refers to The Industrial Psychologist for comment on loss of earnings.
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7.1. PRE-ACCIDENT SCENARIO LOSS OF EMPLOYMENT AND EARNINGS

Employability and earning capacity of an individual should be assessed against criteria impacting individual work performance, such as: cognitive, emotional (mood, drive, motivation) and physical variables. The combination of the person's cognitive, emotional and physical strengths and competencies as well as external factors (the economy, salary trends, demand for skills, etc.) will determine the employability and earning capacity of an individual.

Career development for the majority of people is characterised by the following career stages, normally present at specific ages (Supers Theory):

“Super argues that occupational preferences and competencies, along with an individual’s life situations, all change with time and experience. Super developed the concept of vocational maturity, which may or may not correspond to chronological age: people cycle through each of these stages when they go through career transitions”.

Stage	Age	Characteristics
Growth	Birth-14	Development of self-concept, attitudes, needs and general world of work
Exploration	15-24	"Trying out" through classes, work hobbies. Tentative choice and skill development
Establishment	25-44	Entry-level skill building and stabilisation through work experience
Maintenance	45-64	Continual adjustment process to improve position
Decline	65+	Reduced output, prepare for retirement

Retirement

The Sanlam Benchmark Survey (2019, pg. 08) reveals the following statistics in terms of retirement:

Normal Retirement age for new entrants (males and females) (Sanlam Survey 2019)	
60 years	21.0%
62 years	3.0%
63 years	15.0%
65 years	58.0%

66 years and older	2.0%
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Many South Africans are faced with the problem of not attaining a level of financial security, sufficient to accommodate retirement. According to 'The Retirement Industry Newsletter - IRFA Annual Conference', 2% of the overall population will be able to retire carefree and with the same living standard pre-retirement (by Magnus Heystek Moneyweb, 21/06/2017). Further evidence is noted by Sanlam research surveys (2018), "Retirement fund members are still no closer to moving towards their desired financial outcomes, where they are able to maintain their pre-retirement standard of living". Therefore, the above research would suggest that South Africans face financial insecurity upon retirement, and many citizens are likely not ready to retire.

It is noted that qualifying persons (specifically those in low income groups) qualify for a government pension grant, do consider retirement from the age 60. The majority of these people are then however forced to make an effort to supplement their pension income with earning activities for as long as their health would allow.

7.1.2 PRE-ACCIDENT EMPLOYMENT & EARNINGS SCENARIO

At the time of the accident in November 2024, Mr. Motaung was 60 old. He was reportedly working as a General Worker and Team Leader working employed by Mr Magubane. He stated that his duties included utilising machinery to repair the road and patch pothole.

In terms of pre-morbid health, Mr. Motaung stated that he was involved in a pedestrian vehicle accident when he was young and does not recall the date.

Educationally, Mr. Motaung's reported completing Grade 11 level of education. He reported that he has obtained a brick laying certificate. Furthermore, he stated that he had a Code C1 driver's licence and he has no computer skills.

From a career progression perspective:

- Mr. Motaung would have approximately 05 years until reaching retirement age of 65.
- His work history reveals work experience as a brick layer, general worker and team leader.
- In terms of career aspiration, Mr. Motaung reported that he aspired to open his own construction company.
- When considering Mr. Motaung's age, educational background and work experience; he would have likely remained competitive for unskilled to semi-skilled occupations in the open labour market.

Having regard of the above, the writer postulates that had the accident not happened in November 2024,

- Mr. Motaung would have likely continued with employment as a General Worker and Team Leader, reportedly earning R350.00 per day working five days a week. *(no proof of income received; deferment is made to factual information).*
- His reported income of R350.00 per day equates to R1 750.00 per week and would amount to approximately R91 000.00 per annum.
- For benchmarking purposes, the Koch Non-Corporate Sector salary scales are considered.
- His reported earnings ranged between the median and the upper quartile of the unskilled worker in the non-corporate sector (Koch 2024)
- Collateral information obtained from Mr. Magubane indicates that the project under which Mr. Motaung was employed concluded in February 2025. It is therefore reasonable to assume that he would have remained employed in this capacity until the end of that contract period.
- When considering his expressed intention to continue working in the industry, in addition to his extensive formal and informal experience, it is plausible that following the cessation of his employment contract in February 2025, Mr. Motaung could have secured intermittent, short-term contract work within 6-12 months.
- In such capacity, his earnings would likely have ranged within the median earnings of unskilled workers in the non-corporate sector, with increases subject to annual inflation, until reaching retirement age of 65 years.

Table: Non-Corporate sector (The Quantum Yearbook 2024 - R. J. Koch)

Quartile/Level per annum	Unskilled	Semi-skilled
Lower	R27 600	R39 000
Median	R49 800	R83 000
Upper	R110 000	R218 000

Table: Non-Corporate sector (The Quantum Yearbook 2026 - R. J. Koch)

Quartile/Level per annum	Unskilled	Semi-skilled
Lower	R29 500	R41 600
Median	R53 200	R89 000
Upper	R118 000	R233 000

7.2. POST-ACCIDENT LOSS OF EMPLOYMENT AND EARNINGS SCENARIO

As a result of the accident in November 2024, Mr. Motaung sustained injuries as reflected in section 5.2. Consequently, Mr. Motaung reported the following post-accident complaints: he experiences pain on his neck, right shoulder, back, rib cage, chest and right leg exacerbated by cold weather, he further stated that his feet swell and he struggles to wear closed shoes as well as to lift and carry heavy objects.

It has been over a year since the accident and Mr. Motaung continues to experience post-accident difficulties. At the time of the accident, he was employed as a General worker and Team Leader.

In terms of post-morbid employment history, Mr Motaung reported that he did not return to work as he would not be able to cope with work demands; as a result of his inability to lift and carry heavy objects due to pain on his back, rib cage, chest, neck, right shoulder and right leg exacerbated by cold weather. At the time of finalising the report, Mr Motaung reported that he remains unemployed.

The experts have reported the following regarding Mr. Motaung's post-accident injuries:

According to the Orthopaedic Surgeon Report by Dr. S. K. Mafeelane: *"X-rays were done by Drs. Mkhabele and Indunah Inc. on 12 January 2026 and were reported as follows: Linear lucency with sclerotic margins and marginal osteophytes in the neck of the humerus. There is associated bony proliferation noted around the old fractures line. The humeral head is rotated clockwise within the glenohumeral joint. Significant cortical irregularity and bony proliferation noted in the base of the 5th metatarsal bone suggestive of chronic injury. Joint space narrowing of the cuboideonavicular and 4th metatarsophalangeal joint suggestive of osteoarthritic (secondary) changes. He has 12% whole person impairment."*

According to the Cardiothoracic Surgeon report by Prof. R. F. Chauke: *"Mr Motaung sustained acute thoracic injury–related morbidity for a period of at least three (3) months following the accident. The prognosis is good. WPI is 17%."*

According to the Neurologist report by Dr. B. Ngele: *"Neurologically, although he sustained a mild traumatic brain injury with ongoing headaches and sleep disturbance, there is no objective evidence of significant cognitive impairment. Mr Thabiso Jacob Motaung has shown a stable neurological recovery following his mild traumatic brain injury, with preserved cognition, intact cranial nerves, and normal motor and sensory function, and no evidence of permanent focal neurological deficit or progressive decline. However, he continues to experience persistent post-traumatic headaches and sleep disturbance, which may remain chronic and affect his endurance and quality of life."*

According to the Clinical Psychologist report by Dr. T. Sibanda: *"THE QEEG is inclined towards deviation from the norm indicating neurocognitive deficits. In the assessment, the claimant reported among other experiences, low self esteem, anxiety and depression. The PTSD symptoms were present."*

According to the Occupational Therapist report by L. Tau: *“the residual work capacity to engage in sedentary to light strength requirement workloads, with the exclusion of frequent to constant above shoulder and overhead level reach action, as well as exclusion of frequent to constant below knee level work.”*

7.2.1 Loss of Employment Capacity & Earning Capacity

Generally, it can be accepted that an individual with impairment or disability will be disadvantaged in the open labour market, to a lesser or greater extent in respect of: competitiveness with their uninjured counterparts; efficiency, effectiveness and productivity; lack of motivation and drive to succeed in business or career functioning; emotional functioning; absence from work; decrease in occupational choices.

The writer notes that Mr. Motaung’s highest reported level of education is Grade 11 level of education with a bricklayer certificate. His employment profile suggests that he would be dependent on adequate physical capabilities to execute work tasks effectively. The expert opinions are considered for case formulation.

From a physical capacity perspective, the Orthopaedic Surgeon opined he has great difficulty with prolonged walking, carrying and lifting heavy objects and he has great difficulty doing overhead activities. The Cardiothoracic Surgeon stated that is expected to continue experiencing intermittent episodes of chest pain and generalised musculoskeletal pain. He is expected to experience functional limitations which include reduced tolerance for physical exertion and difficulty with activities requiring upper body strength or endurance. His limitations may manifest as decreased endurance and productivity and need for periodic absenteeism during pain exacerbations.

From the neurological capacity perspective, it is noted from the Neurologist that Mr. Motaung experiences persistent pain, fatigue, and reduced tolerance for sustained effort may indirectly limit his ability to participate in future training or skills development. He may be limited by chronic musculoskeletal pain and physical restrictions from

associated polytrauma, resulting in reduced mobility, an antalgic gait, and diminished tolerance for strenuous activity.

From the neuropsychological perspective, it is noted from the Clinical Psychologist that Mr. Motaung experiences cognitive symptoms: cloudiness, feeling slowed down, concentration problems and difficulty to remember and the emotional symptoms present were irritability, sadness, nervousness and fluctuating mood.

In terms of work capacity perspective, the Occupational Therapist opined that Mr. Motaung's chest pains and shortness of breath will lead to decreased stamina in physically demanding tasks. He does not possess the functional capacity to execute occupation as a General Worker on a construction site due to ongoing symptomology in his right upper limb, right foot, chest region, recurring headaches, as well as difficulties with bilateral manual handling of medium to heavy loads. He is now of pension age and not expected to re-enter the employment market.

It is also prudent to consider the open labour market where Mr. Motaung would be competing for employment. According to Statistics South Africa (Stats SA) Quarterly Labour Force Survey (Quarter 4: 2025), the official unemployment rate is 31.4% Thus, in a saturated, highly competitive open labour market, it can be expected that he may struggle to secure employment.

Having regard of the above, Mr. Motaung is currently suffering from post-accident injuries which have resulted in post-morbid deficits. The writer therefore postulates that Mr. Motaung has experienced a decrease in work capacity as a result of the post-morbid injuries.

7.2.2 Past Loss of Earnings

- At the time of the accident (26th of November 2024), Mr. Motaung stated that he was employed as a General Worker and Team Leader.

- Mr. Motaung reported that he was hospitalised for approximately one month and recuperated at home for another month.
- He indicated that he was not remunerated whilst in recuperation. Therefore, past loss of earnings may be applicable.
- Compensation can be considered for medical expenses incurred during hospitalisation and recuperation period.

7.2.3 Future Loss of Earnings

- Mr Motaung reported that he did not return to work as he would not be able to cope with work demands; as a result of his inability to lift and carry heavy objects due to pain on his back, rib cage, chest, neck, right shoulder and right leg exacerbated by cold weather.
- At the time of finalising the report, Mr Motaung reported that he remains unemployed and receiving old age pension grant.
- Of note, the Occupational Therapist stated that: *“due to ongoing symptomology in his right upper limb, right foot, chest region, recurring headaches, as well as difficulties with bilateral manual handling of medium to heavy loads, the claimant does not possess the functional capacity to execute occupation as a General Worker on a construction site.”*
- Taking into account the opinion above, the writer notes that Mr. Motaung is functioning in a diminished capacity in his injured state and it is justifiable that he did not resume his pre-accident work.
- It is prudent to consider that Mr. Motaung’s vocational and employment history suggests that he would be reliant on adequate physical capabilities to optimally function in the work environment.
- However, according to the Orthopaedic and Cardiothoracic Surgeon, he experiences chronic musculoskeletal pain, reduced upper limb function, difficulty with overhead tasks, and decreased tolerance for physical exertion. It is accepted that these restrictions would limit his work choices and he would struggle to secure employment.
- Furthermore, his reduced endurance, intermittent chest pain, and need for rest periods would likely result in decreased productivity and increased absenteeism.

- Moreover, the headaches, fatigue, slowed processing, concentration difficulties, and memory challenges would further affect his work rate and reliability.
- While he may be considered for a team leader position, which is less physically demanding, the role still requires on-site presence, active supervision, coordination of tasks, and effective communication and decision-making. In light of his physical limitations, reduced endurance, ongoing pain, and cognitive and psychological symptoms, Mr. Motaung would likely experience difficulty meeting the duties associated with such a position.
- Noting all the information at hand and taking into account that Mr. Motaung is already a recipient of old age pension grant and not expected to re-enter the open labour market as stated by the Occupational Therapist, the writer opines that Mr. Motaung experiences challenges which would preclude him from operating in the open labour market. Therefore, he has been rendered unemployable.

7.2.4 Recommendations

- Future treatments etc. required has been documented in the expert reports; the writer recommends that allowance should be made for these treatments to allow Mr. Motaung's optimal recovery.

8. CONCLUSION

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- In the absence of the accident in question, Mr. Motaung would have been able to increase his earnings as stated in section 7.1.
 - The accident in question left Mr. Motaung with post-accident deficiencies which have compromised his work performance, earning capacity and employability. He is deemed an unequal competitor in the open labour market, when compared to uninjured counterparts.

9. RIGHT TO AMEND

The above recommendations and conclusions are based on the information made available to the writer at the time of the evaluation. The writer therefore reserves the right to amend this report should new information become available.

MIRRIAM MAGETHI

Industrial Psychologist

PS 0122220

Electronically signed & submitted

